UUID:5578086D-1892-4413-ACAD-F5995036204C
TCGA-NK-A5D1-01A-PR

Redacted

Path: Carcinoma, squamous cell, basaloid ICD-O-3 8083/3
CSCF: Carcinoma, squamous cell NOS 8070/3

Site: (R) lung upper lobe C34.1

PATHOLOGY SURGICAL
Results
Status: Edited

Entry Date

Component Results
PATHOLOGY RESULT:

Department of Pathology

Final
*** Supplemental information follows Final Report ***
Clinical Data Repository* This report may not match the original report format

FINAL DIAGNOSIS
A. (Right upper lobe wedge; excision): Invasive poorly differentiated squamous cell carcinoma, basaloid type, grade 3, measuring 2.5 cm, with lymphovascular invasion. Visceral pleura free of tumor.
B. (Level 12R lymph node; biopsy): Fragments of lymph node with metastatic poorly differentiated squamous cell carcinoma with extracapsular extension.
C. (Right upper lobe; lobectomy): Residual invasive poorly differentiated squamous cell carcinoma, basaloid type, grade 3, measuring 1.3 cm. Bronchial and vascular margins free of tumor. Visceral pleura free of tumor. Stage: pT2AN1PL0. See note.
D. (4R lymph node; biopsy): Fragments of benign lymph node.
E. (2R lymph node; biopsy): Fragments of benign lymph nodes.
F. (Level 8 lymph node; biopsy): Fragments of benign lymph nodes.

INTRAOPERATIVE CONSULTATION

A. FROZEN:RIGHT UPPER IOBE WEDGE: (A91, TP, smear): Poorly differentiated squamous cell carcinoma.

NOTE

The size of the tumor of 3.8 cm reflects the tumor present in both wedge biopsy and lobectomy. Special studies show that tumor cells are positive for P63 and CK 5, negative for TTF-1, synaptophysin, and chromogranin, supporting above diagnosis. Immunohistochemistry controls are examined and show appropriate reactivity.

Lung Cancer Summary

Specimen type: Lung

Procedure: Wedge biopsy and lobectomy

Specimen integrity: Intact

Tumor location: Right upper lobe

Tumor focality: Unifocal

Tumor size: 3.8 cm

Histologic type: Squamous cell carcinoma, basaloid type

Histologic grade: Grade 3

Treatment effect: N/A

Status of margins:

- Bronchial: Free of tumor
- Vascular: Free of tumor
- Parenchymal: Free of tumor
- Pleural: Free of tumor

Visceral/Parietal pleural invasion: Absent

Lymphatic vascular invasion: Present

Lymph node status: Positive for metastatic tumor at level 12R, negative at level 2R, 4R, and 8

Extension into other structures: Absent

Non-neoplastic pathology: N/A

TMN stage: pT2AN1PL0

GROSS DESCRIPTION

A. The specimen is labeled right upper lobe wedge. Received fresh for frozen section, is a 9 x 6.1 x 2.2 cm lung wedge with a 9cm staple line. The specimen is previously opened and reveals a 2.5 x 2.2 x 1.8 cm white-tan, irregular, firm tumor. The tumor is 0.1 cm to the nearest margin underneath the staple line. The staple line is removed and the exposed parenchyma is inked black. Representative sections of the tumor are submitted for frozen section with the remnant put in cassette A91. A touch prep is performed; more cassettes are submitted as follows: A1 through A3 sections of tumor with the nearest margin, A4 lung parenchyma underlying the staple line, A5 unaffected lung.

B. This specimen is labeled level XII R. lymph node. The specimen is received fresh and consists of multiple anthracotic lymph node fragments

measuring in aggregate 1.1 x 0.9 x 0.2 cm. The specimen is entirely submitted in cassette B1.

C. The specimen is labeled right upper lobe. Received without fixative is a 145 g, 17.0 x 7.0 x 2.7 cm lobe of lung with stapled bronchial and vascular margins. There is a 13.5 cm in length stapled line adjacent to the hilum and a 14.0 cm in length stapled line on the opposite side. A palpable mass is felt and the overlying pleura is inked green. The staple line adjacent to the pleura is removed and the exposed parenchyma is inked yellow. The staple line on the opposite side is removed and the exposed parenchyma is inked black. Sectioning shows a 1.3 x 1.3 x 1.2 cm firm tan mass that grossly abuts the anti-hilar staple line (black) and is 0.2 cm subjacent to the overlying pleura (green), is 1.0 cm from the hilar staple line (yellow) and is 1.5 cm from the bronchial margin. Additionally, there is a 1.0 x 0.9 x 0.7 cm poorly-defined hemorrhagic subpleural nodule located greater than 5.0 cm from the bronchial margin and greater than 2.0 cm from both staple lines. The remainder of the specimen consists of spongy red lung parenchyma without nodules or other lesions.

C1 - bronchial margin, C2 - vascular margins, C3 and C4 - mass with overlying anti-hilar staple margin (black) and pleural (green), C5 - additional sections of nearest pleural and anti-hilar staple margins, C6 - hilar staple margin (yellow) closest to firm nodule, C7 - hemorrhagic nodule, C8 - representative sections of unremarkable lung.

D. The specimen is labeled 4R lymph node. The specimen is received fresh and consists of multiple tan-pink and anthracotic lymph node fragments measuring in aggregate 1.1 x 0.9 x 0.2 cm. The specimen is entirely submitted in cassette D1.

E. This specimen is labeled 2R lymph node. The specimen is received fresh and consists of a single portion of anthracotic soft tissue that measures 0.3 x 0.2 x 0.2 cm. The specimen is entirely submitted in cassette E1.

F. This specimen is labeled 8 lymph node. The specimen is received fresh and consists of a single anthracotic portion of lymph node that measures 0.4 x 0.4 x 0.2 cm. The specimen is entirely submitted in cassette F1

SPECIMEN(S) RECEIVED

A: FROZEN:RIGHT UPPER IOBE WEDGE

B: LEVEL 12R LYMPH NODE

C: FRESH-RIGHT UPPER LOBE

D: FRESH- 4R LYMPH NODE

E: FRESH -2R LYMPH NODE

F: FRESH-8 LYMPH NODE

* The above listed tests (if any) were developed and the performance characteristics determined by)epartment of Pathology. These tests have not been cleared or approved for commercial use by the U.S. Food and Drug Administration. The FDA has determined that FDA review is not required for such in-house assays. (21 CFR 809.30(e))

OPERATIVE INFORMATION

DIAGNOSIS: LUNG MASS; PROCEDURE: BRONCHOSCOPY FLEXIBLE FIBEROPTIC,

THORACOSCOPY VIDEO ASSISTED
INTRADEPARTMENT CONSULTATION PATHOLOGIST: QA COMMITTEE
ICD-9(s): 162.3
CPT(s): A: 88307, 88331, 88334, 88334, 88342 ttf1, 88342 syn, 88342 chrm, 88342 generic, 88342 p63, 88365 hpvhr, 88365 hpvlr, 88342 p16
B: 88305
C: 88309
D: 88305
E: 88305
F: 88305
SUPPLEMENTAL REPORTS

Tumor cells are negative for P16, HPV-6, and HPV-16 . Controls are examined and show appropriate reactivity.
The attending pathologist was physically present when this specimen was diagnosed, and actively participated in all key decisions.
Lab and Collection

Result History

PATHOLOGY SURGICA
Pathology

Procedure Location:

Diagnostic discrepancy from provided: tissue is small cell squamous cell carcinoma (No basaloid type seen)

Criteria	Yes	No
Diagnosis Discrepancy	✓	✓
Primary Tumor Site Discrepancy		✓
HIPAA Discrepancy		✓
Prior Malignancy History		✓
Dual/Synchronous Primary Noted		

Case is (circle): QUALIFIED / DISQUALIFIED
Reviewer Initials: [illegible] Date Reviewed: 12/10/13

TCGA Pathologic Diagnosis Discrepancy Form

V4.0

Instructions: *The TCGA Pathologic Diagnosis Discrepancy Form should be completed when the pathologic diagnosis documented on the initial pathology report for a case submitted for TCGA is inconsistent with the diagnosis provided on the Case Quality Control Form completed for the submitted case.*

Tissue Source Site (TSS) _____

Completed By *(Interviewer Name on OpenClinica)* _____

Diagnosis Information

#	Data Element	Entry Alternatives	Working Instructions
1	Pathologic Diagnosis Provided on Initial Pathology Report	Squamous Cell Carcinoma, Basaloid Type	Provide the diagnosis/ histologic subtype(s) documented on the initial pathology report for this case. If the histology for this case is mixed, provide all listed subtypes.
2	Histologic features of the sample provided for TCGA, as reflected on the CQCF.	Small Cell Squamous Cell Carcinoma	Provide the histologic features selected on the TCGA Case Quality Control Form completed for this case.

Discrepancy between Pathology Report and Case Quality Control Form

3	Provide the reason for the discrepancy between the pathology report and the TCGA Case Quality Control Form.	Tumor Sampling. No Basaloid component seen.	Provide a reason describing why the diagnosis on the initial pathology report for this case is not consistent with the diagnosis selected on the TCGA Case Quality Control Form.
4	Name of TSS Reviewing Pathologist or Biorepository Director	_____	Provide the name of the pathologist who reviewed this case for TCGA.

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled.

_____ _____

TSS Reviewing Pathologist or Biorepository Director Date

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled. The Attending Pathologist or the Department Chairman has been informed or is aware of the above discrepancy in diagnoses.

_____ _____

Principal ... ure Date